HCC3 EXAM 3 | COMPLETE STUDY GUIDE

University of West Georgia • Tanner Health School of Nursing
Based on the Exam 3 Learning Map: Sexuality & Reproduction. Approximate emphasis: Sexuality/STIs/Contraception 25 questions; Prenatal/Antenatal/High-Risk Pregnancy 35 questions. Counts may change. Expanded explanations are standard nursing review, not an official answer key.

PART 1 — SEXUALITY, STIs & CONTRACEPTION

Sexuality and age-related sexual expression

Sexuality includes identity, attraction, intimacy, body image, sexual function, relationships, and reproduction. Infants/toddlers explore their bodies; school-age children develop privacy and body awareness; adolescents experience puberty and explore relationships and identity; adults may focus on intimacy, contraception, fertility, and sexual function; older adults may remain sexually active despite menopause, chronic illness, vaginal dryness, or erectile changes.

Nursing: Use nonjudgmental language, maintain confidentiality, ask permission before discussing sexual health, assess consent and coercion, avoid assumptions about age or orientation, and teach STI prevention at every age.

Common STIs: recognition, treatment, teaching

STI	Typical findings	Treatment / key teaching
Chlamydia	Often asymptomatic; dysuria, discharge, pelvic pain	Doxycycline commonly for nonpregnant adults; azithromycin in pregnancy. Treat partners; retest ~3 months.
Gonorrhea	Purulent discharge, dysuria, pelvic pain; may be silent	IM ceftriaxone; add chlamydia coverage if not excluded; partners need evaluation/treatment.
Syphilis	Primary painless chancre; secondary palm/sole rash; later systemic damage	Penicillin G; regimen by stage. Penicillin required in pregnancy; desensitize if allergic.
HSV	Painful vesicles/ulcers; recurrent outbreaks	Acyclovir/valacyclovir control outbreaks, not cure. Avoid sex with lesions.
HPV	Often asymptomatic; warts or cervical changes	Vaccination and cervical screening; certain types linked to cancers.
Trichomoniasis	Irritation, frothy discharge, odor; strawberry cervix possible	Oral metronidazole; treat sexual partners.
HIV	Early flu-like illness or asymptomatic	Antiretroviral therapy; condoms, PrEP/PEP, testing.
Hepatitis B	Often asymptomatic; possible jaundice	Vaccination; evaluate chronic infection for antiviral therapy.
PID	Pelvic pain, cervical motion tenderness, fever	Broad-spectrum therapy often ceftriaxone + doxycycline + metronidazole.

Risk factors: Inconsistent condom use, new/multiple partners, prior STI, partner with STI, substance use, limited screening access, sexual coercion. Untreated chlamydia/gonorrhea may cause PID, infertility, or ectopic pregnancy.

Prevention	Definition	Examples
Primary	Prevent infection	Condoms, HPV/HBV vaccines, PrEP, education
Secondary	Detect early	STI/HIV screening and pregnancy screening
Tertiary	Treat and reduce complications	Antibiotics, partner services, HIV care, PID follow-up

Standard precautions: Hand hygiene for all; gloves for anticipated exposure to body fluids/mucosa/nonintact skin; gown/mask/eye protection by exposure risk; sharps safety and equipment cleaning. HIV and hepatitis B do not automatically require airborne isolation.

Teaching: Chlamydia: abstain until 7-day regimen completed or 7 days after single dose, and partners treated. Gonorrhea: abstain 7 days after treatment and until partners treated. Condoms reduce but do not eliminate HPV/HSV transmission.

Contraceptive methods and selection

Method	Core facts / patient education
Combined oral pill, patch, ring	Estrogen + progestin; adherence required; screen for estrogen contraindications.
Progestin-only pill	No estrogen; timing/missed-pill rules depend on formulation.
Depo-Provera	Injection about every 13 weeks; irregular bleeding, delayed fertility return, bone-density concerns.
Hormonal IUD	Highly effective long-acting contraception; may lighten menses; no STI protection.
Copper IUD	Hormone-free; may increase cramps/bleeding; can serve as emergency contraception.
Implant	Highly effective long-acting progestin; irregular bleeding common.
Condoms	Reduce pregnancy and STI risk; use correctly each time.
Diaphragm	Barrier with spermicide; fit and correct use matter.
Sterilization	Permanent; informed consent; no STI protection.
Emergency contraception	Levonorgestrel ASAP (best within 72 h; benefit to 120 h); ulipristal to 120 h; copper IUD within ~5 days.

Combined hormonal contraindications: Migraine with aura; severe HTN (at least 160/100); high-risk VTE/thrombophilia; current breast cancer; certain liver diseases; early postpartum period; smoking at age 35+ (especially 15+ cigarettes/day).

Interactions: St. John's wort, phenytoin and other enzyme-inducing anticonvulsants, rifampin/rifabutin can reduce some hormonal contraceptive effectiveness. Most routine antibiotics (e.g., amoxicillin, azithromycin, doxycycline) do not. Severe vomiting/diarrhea may interfere with oral pills.

Emergency contraception: It prevents pregnancy, does not end an established pregnancy, and does not protect from STIs. After ulipristal, wait at least 5 days to start/resume hormonal contraception and use appropriate backup protection.

PART 2 — PRENATAL & ANTENATAL

Normal adaptations of pregnancy

System	Expected changes / nursing implications
Cardiovascular	Blood volume rises ~40–50%; increased cardiac output, physiologic dilutional anemia; avoid prolonged supine positioning.
Respiratory	Increased tidal volume and oxygen needs; mild breathlessness may occur; severe dyspnea is abnormal.
Renal	Increased GFR and urinary frequency; assess for UTI.
GI	Nausea, reflux, constipation, slowed motility; small meals, fluids, fiber.
Musculoskeletal	Lordosis, ligament relaxation, shifted center of gravity; fall prevention.
Hematologic	Hypercoagulability increases VTE risk.
Endocrine	Increasing insulin resistance, especially later pregnancy.

Supine hypotensive syndrome: Enlarged uterus compresses inferior vena cava, reducing venous return. Turn laterally (often left) if dizzy or hypotensive.

Pregnancy signs

Category	Examples
Presumptive (subjective)	Amenorrhea, nausea, fatigue, breast tenderness, urinary frequency, quickening.
Probable (objective, not definitive)	Positive hCG, Goodell (soft cervix), Chadwick (bluish cervix), Hegar (soft lower uterus), ballottement, uterine enlargement.
Positive (confirmatory)	Ultrasound embryo/fetus or cardiac activity; fetal movement palpated by trained examiner.

GPTPAL and Nägele's rule

G total pregnancies including current; **T** deliveries at 37 weeks or later; **P** deliveries 20 through 36 weeks 6 days; **A** losses before 20 weeks; **L** living children. Twins count as ONE pregnancy/delivery event, but TWO living children if both survive.

Example: Current pregnancy + miscarriage at 10 weeks + twins at 34 weeks + one infant at 39 weeks, all three children living = **G4 T1 P1 A1 L3**.

Nägele's rule: First day of last menstrual period + 7 days – 3 months + 1 year. LMP January 10, 2026 → EDD October 17, 2026. Assumes 28-day cycle; early ultrasound may refine dating.

Fetal assessment and testing

Test	What to know / nursing interventions
Kick counts	Common teaching: start ~28 weeks per provider; count 10 movements within 2 hours. Decreased/absent movement needs prompt assessment.
Ultrasound	Confirms intrauterine pregnancy, dates pregnancy, checks anatomy, growth, placenta, fluid; no ionizing radiation.
Biophysical profile	NST, breathing, movement, tone, fluid; each 0 or 2. 8–10 reassuring if fluid adequate; 6 equivocal; 4 or less abnormal. Ultrasound-only 8/8 may be reassuring.
Amniocentesis	Ultrasound-guided amniotic fluid sampling. Consent, fetal/maternal monitoring; watch for leaking fluid, bleeding, contractions, infection; RhIG if indicated.
CVS	Placental tissue genetic testing ~10–13 weeks. Does not directly assess open neural tube defects. Watch bleeding/cramping/leakage.
NST	At ≥32 weeks, reactive: ≥2 accelerations ≥15 bpm for ≥15 sec in 20 min; 10x10 commonly before 32 weeks. Position laterally/semi-Fowler; monitor 20–40 min.
CST	Negative: no late decelerations with adequate contractions; positive: late decels with ≥half contractions. Stop oxytocin if excessive contractions/late decels; reposition, assess BP, notify provider.

BPP details: Breathing ≥1 episode of 30 sec; ≥3 gross movements; ≥1 tone episode; adequate fluid often deepest vertical pocket ≥2 cm; reactive NST. Low fluid warrants attention regardless of total score.

PART 3 — HIGH-RISK PREGNANCY

Pregestational vs. gestational diabetes

Feature	Pregestational diabetes	Gestational diabetes
Timing	Diabetes existed before pregnancy	First recognized during pregnancy, not clearly preexisting
Risks	Congenital anomalies, preeclampsia, macrosomia, neonatal hypoglycemia	Macrosomia, shoulder dystocia, preeclampsia, neonatal hypoglycemia
Management	Glucose monitoring, nutrition, insulin as needed	Diet, activity, glucose monitoring; medication if needed
After birth	Persists	May resolve; postpartum OGTT 4–12 weeks

Common glucose targets: Fasting <95 mg/dL; 1-hour post-meal <140; 2-hour post-meal <120 (individualize). Routine GDM screening usually 24–28 weeks. Treat conscious hypoglycemia with 15 g fast carbohydrate, recheck in 15 min. If unable to swallow, use IV dextrose or glucagon as ordered; never give oral glucose to unconscious patient.

Chronic hypertension and hypertensive disorders

Chronic HTN: Present before pregnancy or before 20 weeks; risks include superimposed preeclampsia, abruption, fetal growth restriction, stroke. Pregnancy-compatible agents include labetalol, extended-release nifedipine, methyldopa. **Avoid lisinopril/ACE inhibitors and ARBs.** Low-dose aspirin (often 81 mg/day) is recommended for qualifying high-risk patients, typically initiated 12–28 weeks, ideally before 16.

Preeclampsia: New hypertension after 20 weeks (≥140/90) with proteinuria (≥300 mg/24 h or protein/creatinine ≥0.3) OR relevant end-organ findings. Severe-range BP ≥160 systolic OR ≥110 diastolic. Severe features include thrombocytopenia, renal/liver dysfunction, pulmonary edema, persistent headache or visual symptoms. Edema alone is not diagnostic.

Nursing care: Monitor BP, neuro symptoms, DTRs/clonus, urine output, platelets, creatinine, AST/ALT, fetal status; seizure precautions; administer prescribed antihypertensives and magnesium sulfate when indicated. Avoid excess IV fluids because pulmonary edema can occur; do not routinely give diuretics just for dependent edema.

Eclampsia: Seizure associated with preeclampsia. During seizure: call for help, protect from injury, position laterally if possible, maintain airway, DO NOT restrain or place objects in mouth, record seizure, assess oxygenation. Afterward stabilize mother, assess fetus, administer magnesium as ordered, prepare for delivery when indicated.

Magnesium sulfate: Seizure prophylaxis/treatment, not the primary antihypertensive. Monitor respiratory rate, DTRs, urine output, consciousness. Toxicity: absent reflexes, respiratory depression, cardiac effects. **STOP infusion, call for help, support airway; antidote IV calcium gluconate.**

Hyperemesis gravidarum

Severe persistent vomiting with dehydration, weight loss, electrolyte/nutritional deficiencies. Assess vitals, orthostasis, mucous membranes, I&O, weight, electrolytes. Give isotonic IV fluids and ordered antiemetics (ondansetron/promethazine), advance diet as tolerated. **Provide thiamine before dextrose** in prolonged vomiting/malnutrition to prevent Wernicke encephalopathy.

Cervical insufficiency and cerclage

Painless second-trimester cervical dilation can cause loss or preterm birth. Cerclage is a supporting cervical suture for selected patients. Report contractions, pelvic pressure, bleeding, leaking fluid, fever, foul discharge. Transvaginal cerclage commonly removed around 36–37 weeks unless earlier indicated.

Miscarriage / abortion

Type	Finding
Threatened	Bleeding; cervix closed; pregnancy may continue.
Inevitable	Bleeding/cramping with cervical dilation.
Incomplete	Some products expelled; some retained.
Complete	All products expelled.
Missed	Pregnancy demise without expulsion.
Septic	Loss complicated by infection.

Nursing: Assess bleeding, pain, vitals, hemodynamic status; ultrasound and labs, hemoglobin, infection signs; emotional support; medications or uterine evacuation when indicated. Assess Rh status and RhIG eligibility according to gestational age, guidelines and protocol. Heavy bleeding, syncope, hypotension or fever are emergencies.

Ectopic pregnancy

Implantation outside uterus, usually tube. Risk factors: previous ectopic, PID, tubal surgery, smoking. Findings: missed period, unilateral pain, spotting; **shoulder pain, syncope, hypotension** suggest rupture and internal hemorrhage. Prioritize ABCs, IV access, vitals, blood preparation, urgent surgical evaluation. Stable selected patients may receive methotrexate and serial hCG; teach to avoid alcohol and folic acid supplements unless directed and follow pregnancy-avoidance advice.

Placenta previa vs. placental abruption

Finding	Placenta previa	Placental abruption
Bleeding	Classically painless, bright red	Often painful; blood may be concealed
Uterus	Typically soft/nontender	May be tender, rigid/hypertonic
Diagnosis	Ultrasound placental location	Primarily clinical; ultrasound may miss
Nursing priority	NO digital vaginal exam until excluded; stabilize	Treat hemorrhage, monitor fetal compromise, prepare urgent care

HELLP and DIC

HELLP: Hemolysis, Elevated Liver enzymes, Low Platelets. Look for RUQ/epigastric pain, nausea, malaise, headache; elevated AST/ALT and LDH, thrombocytopenia. Monitor bleeding, BP, renal/liver labs, fetal status; magnesium when indicated; prepare for delivery after stabilization.

DIC: Widespread clotting activation consumes platelets/factors and causes bleeding. Can follow severe abruption, sepsis, major hemorrhage. Findings: oozing IV sites, low platelets/fibrinogen, prolonged PT/aPTT, elevated D-dimer. Treat underlying cause, monitor shock/urine output, prepare ordered blood products, avoid unnecessary invasive procedures.

PART 4 — MEDICATION MASTER REVIEW

Medication	Exam focus
Azithromycin	Chlamydia in pregnancy; GI effects/QT risk
Ceftriaxone	Gonorrhea; check severe beta-lactam allergy history
Doxycycline	Chlamydia/PID; generally avoid in pregnancy; photosensitivity
Penicillin G	Syphilis; necessary in pregnancy; allergy/desensitization
Metronidazole	Trichomoniasis, BV, PID; treat partners for trichomoniasis
Vancomycin	Serious gram-positive infection; selected GBS prophylaxis; renal monitoring
Ampicillin	GBS prophylaxis and selected infections; allergy review
Lisinopril	ACE inhibitor; contraindicated during pregnancy
Propranolol	Beta blocker; monitor HR/BP; masks hypoglycemia signs
Hydralazine	Acute severe HTN in pregnancy; hypotension/tachycardia
Methyldopa	Pregnancy HTN; sedation, mood and liver effects
Magnesium sulfate	Eclamptic seizure prevention/treatment; monitor RR/DTRs/urine
Calcium gluconate	Magnesium toxicity antidote
Betamethasone	Antenatal fetal lung maturation; monitor maternal glucose
Rho(D) immune globulin	Prevents sensitization in eligible unsensitized Rh-negative patients
Folic acid	Neural tube defect prevention; routine 400–800 mcg/day

Medication	Exam focus
Prenatal vitamins	Supplement folate/iron/iodine; avoid excessive preformed vitamin A
Vitamin D	Supplement deficiency per orders
Insulin	Diabetes; hypoglycemia monitoring; changing pregnancy requirements
Metformin	Type 2/GDM in selected patients; crosses placenta
Dextrose	Hypoglycemia; thiamine first if prolonged malnutrition
Glucagon	Severe hypoglycemia; protect airway, reassess glucose
IV normal saline	Volume repletion; avoid overload in preeclampsia
Ondansetron	Antiemetic; constipation/QT considerations
Promethazine	Antiemetic; sedation/hypotension; injection tissue injury
Lorazepam	Selected seizure emergencies; respiratory depression; not first-line eclampsia
Heparin	VTE prevention/treatment; bleeding/platelet monitoring
Aspirin	Low-dose preeclampsia prophylaxis for qualifying patients
Phenytoin	Antiseizure; reduces hormonal contraceptive efficacy
Depo-Provera	Contraception injection ~13 weeks; bone/bleeding/fertility effects
Combined oral contraceptives	Estrogen contraindications, adherence, interactions
Emergency contraceptives	Levonorgestrel, ulipristal, copper IUD; time-sensitive
HPV/HBV vaccines	Prevent infection; defer HPV vaccine during pregnancy
St. John’s wort	Can reduce hormonal contraceptive effectiveness

RhoGAM and emergency medication contrasts

RhIG: Commonly given around 28 weeks to eligible unsensitized Rh-negative patients and within 72 hours after birth of Rh-positive infant; also after indicated sensitizing events. It does NOT reverse established sensitization. Magnesium sulfate prevents seizures; calcium gluconate treats toxicity. Hydralazine can lower severe BP in pregnancy; lisinopril is contraindicated. Betamethasone promotes fetal lung maturity; it does not stop labor.

PART 5 — 15 PRACTICE QUESTIONS & ANSWERS

1. 32 weeks, BP 168/112, headache, blurred vision, RUQ pain: priority?
Answer: Immediate provider notification and severe-preeclampsia precautions. *Rationale:* Severe-range hypertension and organ symptoms require urgent care.

2. Magnesium infusion, absent reflexes, RR 9/min: priority?
Answer: Stop magnesium and initiate emergency assessment. *Rationale:* Toxicity; prepare calcium gluconate.

3. Painless bright-red third-trimester bleeding suggests?
Answer: Placenta previa. *Rationale:* Avoid digital vaginal exam until excluded.

4. Positive pregnancy test, unilateral pain, shoulder pain, hypotension?
Answer: Ruptured ectopic pregnancy. *Rationale:* Likely intra-abdominal bleeding; emergency.

5. Which is contraindicated in pregnancy: methyldopa, lisinopril, hydralazine?
Answer: Lisinopril. *Rationale:* ACE inhibitors can cause fetal injury.

6. Current pregnancy, 9-week miscarriage, 35-week twins, 39-week singleton; 3 living?
Answer: G4 T1 P1 A1 L3. *Rationale:* Twin delivery counts once for preterm, twice for living.

7. Medication for uncomplicated gonorrhea?
Answer: Ceftriaxone IM. *Rationale:* Recommended first-line therapy.

8. St. John's wort with combined oral contraceptive: risk?
Answer: Reduced contraceptive effectiveness. *Rationale:* Enzyme induction decreases hormone exposure.

9. BPP 4/10: action?
Answer: Prompt provider notification and evaluation. *Rationale:* Abnormal fetal surveillance result.

10. RUQ pain, low platelets, elevated AST/ALT?
Answer: HELLP syndrome. *Rationale:* Hemolysis, elevated liver enzymes, low platelets.

11. Late decelerations during oxytocin CST?
Answer: Stop oxytocin, reposition, assess, notify provider. *Rationale:* Suggests uteroplacental insufficiency.

12. Which contraceptive helps prevent STIs?
Answer: Condom. *Rationale:* Barrier protection; IUDs and hormones do not prevent STIs.

13. Prolonged vomiting and malnutrition before IV dextrose?
Answer: Thiamine. *Rationale:* Prevents Wernicke encephalopathy.

14. Positive sign of pregnancy?
Answer: Fetal cardiac activity seen on ultrasound. *Rationale:* Confirms pregnancy.

15. Abruption patient oozing from IV sites?
Answer: Suspect DIC. *Rationale:* Consumption coagulopathy causes abnormal bleeding.

FINAL HIGH-YIELD RECOGNITION

Clue	Think
Painless bright-red bleeding	Placenta previa
Painful bleeding + rigid uterus	Placental abruption
RUQ pain + low platelets + high liver enzymes	HELLP
Bleeding/oozing IV sites + low fibrinogen	DIC
Severe headache + vision changes + high BP	Preeclampsia with severe features
Seizure with preeclampsia	Eclampsia
Absent DTRs on magnesium	Magnesium toxicity
Unilateral pelvic pain + shock	Ruptured ectopic
Painless second-trimester dilation	Cervical insufficiency
Dehydration + severe pregnancy vomiting	Hyperemesis
Positive CST / late decelerations	Uteroplacental insufficiency
Reduced fetal movement	Prompt fetal assessment

Study priority: (1) Emergencies and nursing priorities; (2) fetal testing; (3) high-risk conditions and medications; (4) STIs/contraception; (5) GPTPAL, pregnancy signs, and due-date calculations. Complete Evolve questions/case studies as your course learning map recommends.

Study aid only. Clinical protocols, medication regimens, and guidelines can change; follow assigned course resources and local policy.